[redacted] 71 Y/F

W/[redacted]

UHID P0005082 IPD No. 25/0927

DOA 17/Feb/2026 00:43

DR [redacted]

[redacted] ART

(A UNIT OF A.S. KIDNEY HOSPITAL & STONE CENTRE PVT. LTD.)

CONSULTANT : [redacted]

DATE : 17/02/26 DOA : 17/02/26

BED No. ICU-1 DIET

CURRENT DIAGNOSIS ANEMIA

HBS Ag / HIV / HCV

Allergy

SPECIAL INSTRUCTION
- Chest X RAY PA view due
- 2D echo (Ask)
- USG HIA
- Iron Profile

PROCEDURE DATE
CAG
PTCA
PA line
Venti
IABP
CABG

INSTRUCTIONS
- MONITOR VITALS
- I/O CHARTING
- 2 U PRBC arranged

Balance

MEDICATION	DOSE	FREQ	ROUTE	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	1	2	3	4	5	6	7
INJ. MONOCEF	1GM	12HRLY	IIV																		1:30am	Sugandha					
INJ ACILOC	50MG	12HRLY	IIV																			Sugandha				⊙	
INJ CEMSET	4MG	12HRLY	IIV																			Sugandha				⊙	
INJ. TRANEXA	1GM	SOS	IIV																								
INJ. LASIX	20MG	SOS	IIV																								
INJ. MVI	1AMP	24HRLY	IIV																			Sugandha					
TAB. DROSOME	1TAB	24HRLY	P/O																			Sugandha					

	1	2	3	4	5	6	7
	(1:15)						
BP	144/67	140/70	130/70	130/80	140/80	140/80	130/80
CVP	–	–					
TEMP	98.2F	98.2F					
PULSE	95	96	97	98	97	97	98
SPO_2	99%	98%	96	97	97	96	97
RESP	18	20	18	16	17	16	18
BLOOD SUGAR	NRS (50ml)						
INTAKE IV						H50 (100ml)	
ORAL/RT							
OUTPUT							
URINE			550 ml				
STOOL							
VOMITTING							

GCS/TIME	8	12	16	20	24	4
SENSORIUM						Conf
EYE RESPONSE						4
VERBAL RESPONSE						5
MOTOR RESPONSE						6
PUPIL SIZE						
RIGHT						N
LEFT						N
VOMITTING						No
Pain Score						2/10
Back Care						✓
VENTILATOR						
TV						
PRES						
O_2FIO_2						
PEEP						
RATE						
PEEK PRES						
ABG						
Ph						
PO_2						
PCO						
HCO_3						
BE						

INVESTIGATION	STAT ORDER	BY
CBC, RFT, LFT Blood Gp., HBA1C, CPK / CPK MB ⊗ due. - Stool for occult Blood due.		

Total Intake ..... Ltr. Output ..... Ltr. Balance ..... Ltr.

Remarks Nurse 1 ..... 2 ..... 3 Sugandha RMO 1 ..... RMO 2 ..... RMO 3 Dr. Vikky

EVENT .....

PTO FOR NOTES

14/2/26

1Am

8Am

NIGHT DUTY NOTES

Patient recieved from ENR staff
Vitals are stable & documented
Intake output charting done.
All prescribed medication done.
Patient is stable & conscious
2O PRBC arranged.. No
fresh complaint till now.

[redacted] 71 Y/F
W/o [redacted]
UHID P0005082 IPD No. 25/0927
DOA 17/Feb/2026 00:43
DR. [redacted]

ART

BED No. ICU-1 DIET

[redacted]
(A UNIT OF A.S. KIDNEY HOSPITAL & STONE CENTRE PVT. LTD.)
DATE: 17/2/26 DOA: 17/2/26
CONSULTANT: [redacted] Balance

CURRENT DIAGNOSIS: ANEMIA, HTN, DM, CAD c PTCA
HBS Ag / HIV / HCV
Allergy: N.K

SPECIAL INSTRUCTION
- Chest x-Ray PA view
- 2D echo, USG W/A (ASK)
- Iron Profile (ASK)
- Stool for occult Blood (ASK)

PROCEDURE DATE: CAG, PTCA, PA line, Venti, IABP, CABG

INSTRUCTIONS
- MONITOR VITALS
- S/O CHARTING

* 2 ⊙ PRBC arranged.

MEDICATION	DOSE	FREQ	ROUTE	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	1	2	3	4	5	6	7
INJ. MONOCEF	1GM	12HRLY	IV													[redacted]											
INJ. ACILOC	50MG	12HRLY	IV												[redacted]	[redacted]										[redacted]	
INJ. EMSET	4MG	12HRLY	IV																								
INJ. TRANEXA	1GM	SOS	IV	Stop																							
INJ. LASIX	20MG	SOS	IV																								
INJ. MVI	1AMP	24HRLY	IV																								
TAB. GRIDSOME	1TAB	24HRLY	PO																								
INJ. AVIL	1AMP	SOS	IV																								

GCS/TIME	8	12	16	20	24
SENSORIUM	Conf	Conf	Cons	Cons	Cons
EYE RESPONSE	4	4	4	4	4
VERBAL RESPONSE	5	5	5	5	5
MOTOR RESPONSE	6	6	6	6	6
PUPIL SIZE					
RIGHT	N	N	N	N	N
LEFT	N	N	N	N	N
VOMITTING	No	No	No	No	No

	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	1	2	3	4	5	6	7
BP	130/90	120/80	110/70	120/76	120/70	120/76	120/70	120/80	[illegible]	120/90	120/80	120/80	120/78	120/70	125/75	110/70	120/65	130/90	120/70	120/70	120/74	100/75	100/76	120/70
CVP																								
TEMP	98F	98F	–	–	–	–	99.8F	98.2F		98.6F	–	–	98.2F	–	–	98.2F	–	–	–	–	–	98.1F	–	–
PULSE	98/min	90	88	100	100	100	98/min	90	89/min	80	80	80	82	86	88	88	80	78	80	82	84	80	80	86
SPO2	97%	97%	97%	98%	97	98	94%	95	96	98	98	98	98%	99%	98%	97%	99	97	98	99	98	97%	98%	99%
RESP	18	16	14	16	16	18	16	18	18	19	19	19	20/min	20/min	18/min	18/min	18/min	16	14	16	14	18	18	20
BLOOD SUGAR	172 mg/dl																							
INTAKE IV					PRBC 350ml		PRBC 100ml					NS 100ml											NS 100ml	
ORAL/RT	BV 250ml, Rice 100g				Lunch 200ml, Water 100ml				Tea 50		Juice 20				Milk (100ml)			Soup (100ml)					Tea (100ml)	
OUTPUT																								
URINE				200ml					200					200ml				150ml					300ml	
STOOL																								
VOMITTING																								

VENTILATOR	8	12	16	20	24
TV					
PRES					
O_2FIO_2					
PEEP					
RATE					
PEEK PRES					
ABG					
Ph					
PO_2					
PCO					
HCO_3					
BE					

INVESTIGATION	STAT ORDER	BY
LBC, RFT, LFT, Blood GP, HbA1c, CPK/CPK-MB (R) due.	Stool for occult, - LBC (R) done.	
	- T done	
1 PRBC	Rundown	HS 28-
1 PRBC	—	

Total Intake 650 ml Ltr. Output 550 ml Ltr. Balance Ltr.

Remarks Nurse 1 [redacted] 2 [redacted] 3 [redacted] RMO 1 RMO 2 [redacted] RMO 3 [redacted]

EVENT

PTO FOR NOTES

(m) Duty over

Patient Received in

(n) Duty nursing

staff assess the pt

Condition vital

checked and Recd

(m) Duty medicine dem

Chest xray, 2 Dexo, USG - UA

Due Next Duty nursing

staff over

Duty nursing staff

1 PRBC Running -

on Pralet

8AM TO 2PM

17/2/26

EVENING NOTES

PT. Received from (m) dr

Pt. alert conscious

vital checked and

All medicine given for doctor order

I/O charting maintain

No IV fluid

TROP - I (-VE)

1 PRBC - 7 pm 5 pm - 5 pm

USG WA - done

2D ECHO done + ABP done

mellitus

Stool for occult blood

2 PRBC - urgent

17/2/26

NIGHT DUTY NOTES

8 pm

Patient handover recieved from evening shift staff. vitals are monitored & documented. Intake output charting done. 10 PRBC done

8 am

& 10 PRBC due. USG WA, serum Profile, 2.D echo - Ask to dr. malik. Stool for occult Blood due. No fresh complaint till now. 10 PRBC is on flow

ICU TREATMENT CHART

BED No. ICU-1 DIET Normal Diet (Diabetic)

[redacted]

CONSULTANT: [redacted] Balance

PATIENT NAME [redacted]
ID 85/0927 AGE / SEX 71/F
DATE: 18/2/26 DOA: 17/2/26

CURRENT DIAGNOSIS ANEMIA
HTN | T2DM | CAD & PTCA

HBS Ag / HIV / HCV

Allergy INJ. MVI

SPECIAL INSTRUCTION
- 1 O PRBC done on 17/2/26
- 1 O PRBC due

PROCEDURE DATE
CAG
PTCA
PA line
Venti
IABP
CABG

INSTRUCTIONS
- MONITOR VITALS
- 9/0 CHARTING

MEDICATION	DOSE	FREQ	ROUTE	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	1	2	3	4	5	6	7
INJ. MONOCEF	1GM	12HRLY	IIV	✓																							
INJ. ACILOC	50mg	12HRLY	IIV																								
INJ. GAMSET	4MG	12HRLY	IIV																								
INJ. LASIX	40mg	SOS	IIV																								
TAB. ROSOME	1TAB	24HRLY	P/O																								
TAB METFORMIN	500	OD	P/O																								

	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	1	2	3	4	5	6	7
BP	120/80	130/80	120/70	120/70	120/70	120/76	120/70	120/70	120/60	130/70	130/80	130/60	130/60	134/68	125/70	120/75	120/68	124/66	130/75	132/76	125/72	120/62	135/69	120/80
CVP																								
TEMP	100F	99.6F	98F	98F	98F	—		—	98.2F	—	—	—	—	—	98.2F	—	—	—	—	—	—	—	98.1F	—
PULSE	98	96	90	94	88	84	86	86	86	84	84	84	84	82	84	80	82	80	84	86	80	82	82	84
SPO2	97%	97%	98%	97%	98%	97%	98	97	98	98	98	98	98	97%	98%	97%	97%	98%	97%	96%	97%	98%	99%	99%
RESP	18	18	16	16	18	16	20	18	18	20	20	20	20	18	18	20	20	18	18	22	20	20	20	22
BLOOD SUGAR																								
INTAKE IV	NS 100ml	7:30am PRBC (330ml)				Lactated 500ml	NS 200ml	Tea 100ml					NS (100ml) Dilution PRBC (240ml)		Milk (100ml)								NS (100ml) Tea	
ORAL/RT																				8:20 (100ml)			(50ml)	
OUTPUT																								
URINE				300ml									450ml					250ml						400ml
STOOL																								
VOMITTING																								

GCS/TIME	8	12	16	20	24	4
SENSORIUM	Con	Con	Con	Con	Con	Con
EYE RESPONSE	4	4	4	4	4	4
VERBAL RESPONSE	5	5	5	5	5	5
MOTOR RESPONSE	6	6	6	6	6	6
PUPIL SIZE						
RIGHT	N	N	N	N	N	N
LEFT	N	N	N	N	N	N
VOMITTING	No	No	No	No	No	No
Pain Score	2/10	2/10	2/10	1/10	2/10	2/10
Back Care	✓	✓	✓	✓	✓	✓
VENTILATOR						
TV						
PRES						
O_2FIO_2						
PEEP						
RATE						
PEEK PRES						
ABG						
Ph						
PO_2						
PCO						
HCO_3						
BE						
	1	1	1	1		

INVESTIGATION	STAT ORDER	BY
- 2D echo - USG W/A - Iron profile } Ask	CBC S. Folate Vit B12 Iron profile	dm 19/2
- Stool for occult Blood (3) due.		

Total Intake 1637 ml Ltr. Output 1100 ml Ltr. Balance 537 ml (+ve Balance) Ltr.

Remarks Nurse 1 [redacted] 2 [redacted] 3 [redacted] RMO 1 [redacted] RMO 2 [redacted] RMO 3 [redacted]

VENT

PTO FOR NOTES

(M) Duty OVER

Patient Received in (M) Duty Nursing STAFF Assess the Patient Condition vital checked and Recorded (M) Duty Medicine done

PRBC II Unit Hold 1A fever 8 Am 100 F° given in PCM 1Gm IV STAT Next Duty Nursing STAFF OVER TO (E) Duty

Sri Balaji
8 Am TO 2 Pm
18/2/26

Evening

Pt Received from (M) duty Staff. Pt vital checked and Received Pt ~~RBS~~ I/O Chart

II PRBC done. S/D Allowed all medication given

No fresh complaint

- BT & Hb at 1:30 pm stop 2:15 pm

[redacted]

18/2/26

NIGHT DUTY NOTES

8pm Patient handover received from evening duty staff. Vitals are monitored & documented. Intake output charting done. All Prescribed medication done.

8am 2 O PRBC done. Patient is taking Diabetic Normal Diet. Stool for occult Blood (S) due. No fresh complaint till now.

[redacted]

[redacted] CHART

BED No. 9W-1 DIET Diabetic Normal diet

[redacted] DATE: 19/2/26 DOA: 17/2/26

CONSULTANT: [redacted] Balance

CURRENT DIAGNOSIS ANEMIA / HTN / T2DM / CAD c PTCA

HBS Ag / HIV / HCV

Allergy c INJ. MVI

SPECIAL INSTRUCTION
- 2D Echo
- USG WA } Ask

PROCEDURE DATE
CAG
PTCA
PA line
Venti
IABP
CABG

INSTRUCTIONS
- MONITOR VITALS
- I/O CHARTING

	M		E		N																							
MEDICATION	DOSE	FREQ	ROUTE	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	1	2	3	4	5	6	7
23 INJ MONOCEF	1gm	12HRLY	IV	[redacted]												•											
INJ. ACILOC	50mg	12HRLY	IV											•												•	
INJ. EMSET	4mg	12HRLY	IV											•												•	
INJ. LASIX	20mg	SOS	IV																								
TAB. DIOSDME	1 TAB	24HRLY	P/O			[redacted]																					
TAB. METFORMIN	500mg	24HRLY	P/O	[redacted]																							
BP				130/70	120/70	Refusal		110/80	120/80	110/70	~~80/40~~																
CVP				—		Shortness																					
TEMP				98.4	98.4			98.4	98.4	98.4	98.4																
PULSE				88	80																						
SPO2				97%	97%	Room		97	98	97	98																
RESP				18	16	102		18	19	16	14																
BLOOD SUGAR																											
INTAKE IV			NS 50ml							Lunch 1120																	
ORAL/RT		200	BF 200							Lunch																	
OUTPUT																											
URINE					✓																						
STOOL																											
VOMITTING																											

GCS/TIME	8	12	16	20	2
SENSORIUM	Con				
EYE RESPONSE					
VERBAL RESPONSE					
MOTOR RESPONSE					
PUPIL SIZE					
RIGHT					
LEFT					
VOMITTING					
Pain Score					
Back Care					
VENTILATOR					
TV					
PRES					
O_2FIO_2					
PEEP					
RATE					
PEEK PRES					
ABG					
Ph					
PO_2					
PCO					
HCO_3					
BE					

INVESTIGATION	**STAT ORDER**
- ~~Stool for occult Blood~~ due (R) due + CBC, ~~Vit B12, Serum~~ Profile, S. Folate (R) due.	

Total Intake 1580 ml Ltr. Output 1400 ml Ltr. Balance 180 ml (+ve) Ltr.

Remarks Nurse 1 [redacted] 2 [redacted] 3 RMO 1 DR RMO 2 RMO 3

EVENT

PTO FOR

(m) Duty OVER

Patient Received to
(n) Duty nursing staff
Assess the patient
Condition vital checked
and Recorded
(m) Duty medicine done
Due lab (P)
Stool for occult
Blood (S)
2PRBC done.
Next Duty nursing
STAFF OVER
(E) Duty nursing
STAFF RNS one
Time checking
Next Duty
ombatch
8AM TO 2hr
19/2/26

19/2/26
Evening duty reports
2pm-8pm

- Received from (E) duty staff at 2pm from Both ompity
- Pt's Conscious and oriented GCS 15
- All drs medicine given done odd
- 2PRBC given
- Today CBC, S. Iron, S. Folate, VitB12 (N) done
- Stool for occult Blood (P) collected (negative)
- Pt no fresh complaint
- Pt's is [illegible]